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Case Number: 25AVCV01354 Hearing Date: September 17, 2026 Dept: A14

SUPERIOR COURT OF THE STATE OF CALIFORNIA

COUNTY OF LOS ANGELES – NORTH JUDICIAL DISTRICT

MICHAEL REILLY, individually and as Successor-in-Interest of the Estate of Amelia Reilly; and CASSANDRA REILLY, an individual,

Plaintiffs,

v.

YASSER E. SADEK, D.D.S.; Y SADEK, DDS, INC., a California corporation dba Palmdale Children's Dental; THIEN C. PHAM, D.D.S.; and DOES 1 through 25, inclusive,

Defendants.

Case Number 25AVCV01354

[TENTATIVE]

RULING ON MOTION

Date of Hearing: September 17, 2026

Dept. A-14

Judge William H. Forman

I. Background

Plaintiffs Michael Reilly ("Mr. Reilly"), individually and as Successor-in-Interest of the Estate of Amelia Reilly ("Estate"), and Cassandra Reilly ("Mrs. Reilly") (together "Plaintiffs") move the Court for leave to file a Second Amended Complaint ("SAC") to allege punitive damages.

Plaintiffs allege that on January 15, 2025, five year old Amelia Reilly ("Amelia"), daughter to Mr. and Mrs. Reilly, was scheduled for a dental procedure for composite restoration of cavities in two teeth and conscious sedation. Mrs. Reilly signed a consent form for the procedure and inhalation of nitrous oxide on December 13, 2024. Plaintiffs assert that Amelia had a known diagnosis of Williams-Beuren Syndrome ("Williams Syndrome") noted in her chart, and that she had been receiving dental treatment from Defendant Yasser E. Sadek, D.D.S. ("Sadek") at Y Sadek, DDS, Inc., dba Palmdale Children's Dental ("PCD") for at least two years on the date of the procedure. Plaintiffs characterize Williams Syndrome as a congenital, multisystem disorder that causes developmental, physical, and mental abnormalities, and that cardiac and renal involvement can be severe, rendering general anesthesia a significant risk.

On January 15, 2025, Sadek recommended a tooth extraction and recommended general anesthesia to Mr. Reilly, who asserts he consented based on Sadek's representations and was unaware of the associated risks. The anesthesia was administered by Thien C. Pham, D.D.S. ("Pham").

By the time the procedures were completed by Sadek, Amelia's condition was deteriorating. Paramedics were called and arrived at approximately 8:33 a.m., but were sent away by Defendants. Defendants removed the IV line, blood pressure cuff, and cardiac monitoring. At approximately 8:45 a.m., Amelia stopped moving. After 15 minutes, Defendants reapplied the cardiac leads and attempted to insert a new IV line. Sadek called paramedics to return. Defendants began performing chest compressions. The paramedics arrived at 9:07 a.m. and took Amelia to the Antelope Valley Medical Center emergency room, where she was pronounced dead at 10:25 a.m.

Plaintiffs allege Defendants' negligence in administering the anesthesia caused Amelia's death.

On October 13, 2025, Plaintiffs filed a Complaint against Defendants Sadek, PCD, and Pham (collectively "Defendants"), asserting nine causes of action for (1) Wrongful Death, (2) Negligence – Bystander, (3) Survivor Action – Negligence, (4) Survivor Action – Lack of Informed Consent, (5) Survivor Action – Battery, (6) Survivor Action – Negligence Misrepresentation, (7) Survivor Action – Fraud by Intentional Misrepresentation, (8) Survivor Action – Fraud by Concealment, and (9) Survivor Action – Constructive Fraud.

On December 8, 2025, Plaintiffs filed their First Amended Complaint ("FAC") asserting the same causes of action against the same Defendants.

On July 17, 2026, the Court granted Plaintiffs' ex parte application for leave to file an oversized memorandum of points and authorities not exceeding 25 pages and to file a reply memorandum not exceeding 15 pages.

On July 30, 2026, Plaintiffs filed their Motion for Leave to Amend the FAC to allege punitive damages.

On September 3, 2026, Sadek and PCD together, and Pham separately, filed their Oppositions.

On September 10, 2026, Plaintiffs filed their Reply.

II. Evidentiary Objections

Pham's Evidentiary Objections – Pham objects to the following evidence submitted by Plaintiffs in support of their Motion:

1. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 28, Page 4:25-5:2 – "According to Dr. Pham's anesthesia records from Amelia Reilly's dental procedure, he used both propofol and ketamine. These drugs are relatively contraindicated in children with Williams Syndrome like Amelia Reilly. Use of these specific induction drugs on Amelia Reilly was below the standard of care. Not knowing that these drugs were dangerous to a child with Williams Syndrome, like Amelia Reilly, was wanton, reckless and grossly negligent, in knowing disregard of the health and safety of this child."
2. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 42, Page 7:1-6 – "In my opinion, it was wanton, reckless, despicable (in conscious disregard of the health and safety of the patient) and grossly negligent to not perform adequate CPR immediately on this child when she became bradycardic again. Per the depositions of Dr. Pham and Dr. Sadek, both were certified in Pediatric Advanced Life Support (PALS). Per the video, Dr. Pham did not instruct Dr. Sadek or the dental assistant to commence compressions at an adequate rate or duration until at least 8 minutes after reapplying the cardiac leads, which showed her heart rate deteriorating again."
3. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 52, Page 8:8-9 – "In my opinion, the anesthesia records created by Dr. Pham contained false and misleading information intended to obfuscate negligence after the fact."
4. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 53, Page 8:10-22 – "Based upon my education, training, experience, and my review of all the information, it is my opinion that Dr. Pham's actions and inactions constitute a wanton and reckless disregard of the rights and safety of this five-year-old child."

Pham argues these statements lack foundation for opinion testimony under Evidence Code section 803, assume facts not in evidence, constitute inadmissible hearsay pursuant to Evidence Code section 1200, are conclusory and speculative, constitute inadmissible expert opinion under *Kelley v. Trunk* (1998) 66 Cal.App.4th 519, 524, are irrelevant under Evidence Code section 210, and are more prejudicial than probative under Evidence Code section 352. Pham also argues these statements ignore the actions taken by Pham during the second incident and prejudicially ignores the training of Sadek and the Dental Assistant, who may intervene on their own, and places responsibility on Pham for actions and decisions of others present with the same knowledge.

Plaintiffs oppose Pham's objections on the grounds that (1) Tom supplies the foundation for his medication-selection opinion in ¶¶ 24-28, that a recommendation need not amount to an absolute prohibition of a medication before it may be used to support an expert's opinion that use was contraindicated and fell below the standard of care, and that Pham's arguments concern the opinion's weight, not its admissibility, (2) *Kelley* is inapposite because Tom identifies facts, literature, and reasoning supporting his opinions, (3) Plaintiffs do not rely on Tom's final sentence to establish Pham's subjective state of mind, (4) Tom's qualifications and materials reviewed support his opinion concerning the need for immediate CPR and his description of the documented response, (5) that Sadek or the Dental Assistant ("DA") could have acted independently does not eliminate Pham's responsibility under Business and Professions Code section 1646.1(e)(3)(B), (6) Pham does not identify any particular hearsay statement or unfair prejudice, and (7) Tom's summary assessment of the severity of medical departures explained in ¶¶ 14-52 is not objectionable merely because it embraces an ultimate issue, the opinion is supported by patient-specific facts and medical reasoning, and it is not offered as expert testimony establishing Pham's subjective intent or direct the Court's punitive damages determination.

Pham's objections are SUSTAINED in part as to the following specific language:

1. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 28 – "Not knowing that these drugs were dangerous to a child with Williams Syndrome, like Amelia Reilly, was wanton, reckless and grossly negligent, in knowing disregard of the health and safety of this child."

2. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 42 – "In my opinion, it was wanton, reckless, despicable (in conscious disregard of the health and safety of the patient) and grossly negligent to not perform adequate CPR immediately on this child when she became bradycardic again."

3. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 52 – "intended to obfuscate negligence after the fact."

4. Declaration of James W. Tom, D.D.S., MS, DADBA, ¶ 53, Page 8:10-22 – "Based upon my education, training, experience, and my review of all the information, it is my opinion that Dr. Pham's actions and inactions constitute a wanton and reckless disregard of the rights and safety of this five-year-old child."

Pham's objections are otherwise OVERRULED.

Sadek and PCD's Evidentiary Objections – Sadek and PCD object to the following evidence submitted by Plaintiffs in support of their Motion:

1. Declaration of Joseph Renzi, D.D.S., ¶¶ 16-17, Page 3:22-25 – Dr. Sadek testified in his deposition that his website incorrectly labels him as a pediatric dentist, and he acknowledged the misrepresentation of his credentials. Dr. Sadek's representations to the Reillys and on his website that he was a pediatric dentist were false and an intentional misrepresentation of his qualifications."

2. Declaration of Joseph Renzi, D.D.S., ¶ 18, Page 4:3-5 – "Dr. Sadek not only violated the law, but he fraudulently misrepresented his qualifications to treat a child like Amelia Reilly in willful and conscious disregard for the health and safety of Amelia Reilly."

3. Declaration of Joseph Renzi, D.D.S., ¶¶ 39-40, Page 6:9-14 – "[Dr. Sadek's] failure to be present throughout the procedure was reckless and wanton and placed Amelia Reilly at extreme risk in conscious disregard of her health and safety....It was grossly negligent not have three people in the room during the entire procedure and especially during this emergency."

4. Declaration of Joseph Renzi, D.D.S., ¶ 42, Page 6:50-23 – "Under Business & Professions Code §1680(z)(1)(C) a dentist is required to report to the Medical Board the removal of any patient to whom general anesthesia to a hospital or emergency center for medical treatment. Dr. Sadek could avoid this reporting requirement by not allowing the EMS to transport Amelia Reilly when she became bradycardic the first time."

5. Declaration of Joseph Renzi, D.D.S., ¶ 43, Page 7:1-3 – "The lack of immediate and consistent CPR from Dr. Sadek or his dental assistant, who were both Pediatric Advanced Life Support (PALS) certified providers, was egregious and wanton, in conscious disregard for the health and safety of Amelia Reilly."

6. Declaration of Joseph Renzi, D.D.S., ¶ 45, Page 7:13-15 – "By not performing this simple internet search, Dr. Sadek was reckless and placed Amelia Reilly at extreme risk of death in conscious disregard for her health and safety."

7. Declaration of Joseph Renzi, D.D.S., ¶ 47, Page 7:19-21 – "Based upon my education, training, experience, and my review of all of the information, it is my opinion that Dr. Pham's actions and inactions constituted a wanton disregard of the rights and safety of this five year-old child."

Sadek and PCD argue these statements lack foundation for opinion testimony under Evidence Code section 803, assume facts not in evidence, constitute inadmissible hearsay pursuant to Evidence Code section 1200, are conclusory and speculative, constitute inadmissible expert opinion under Kelley v. Trunk (1998) 66 Cal.App.4th 519, 524, are irrelevant under Evidence Code section 210, and are more prejudicial than probative under Evidence Code section 352. Sadek and PCD also

argue as to objection No. 7 that the paragraph names Pham as a responsible actor and should not be considered in connection with allegations asserted against Sadek. As to all requests, Sadek and PCD argue the opinions are beyond the scope of expert opinions and should not be considered.

Plaintiffs oppose these objections on the grounds that (1) Renzi's characterization of Sadek's website as a "misrepresentation" is used descriptively and based on Sadek's deposition testimony and is not inadmissible merely because it overlaps a disputed issue under Evidence Code section 805, (2) Kelley is inapposite because Renzi's opinions rest on stated qualifications and facts, not bare conclusion, (3) Sadek does not identify any particular hearsay statement or Section 352 prejudice warranting exclusion, (4) Plaintiffs do not rely on ¶ 18 to establish ultimate legal or punitive conclusions, and (5) Renzi has a reasoned factual and professional basis for his opinion concerning the required team and seriousness of the departure and Sadek's objection concerns weight of the opinion, not admissibility. Plaintiffs do not object to exclusion of Nos. 2 or 4.

Sadek and PCD's objections are SUSTAINED in part as to the following specific language:

1. Declaration of Joseph Renzi, D.D.S., ¶ 17 – "and an intentional misrepresentation of his qualifications."
2. Declaration of Joseph Renzi, D.D.S., ¶ 18 – "Dr. Sadek not only violated the law, but he fraudulently misrepresented his qualifications to treat a child like Amelia Reilly in willful and conscious disregard for the health and safety of Amelia Reilly."
3. Declaration of Joseph Renzi, D.D.S., ¶¶ 39-40 – "was reckless and wanton... in conscious disregard of her health and safety" and "It was grossly negligent not have three people in the room during the entire procedure and especially during this emergency."
4. Declaration of Joseph Renzi, D.D.S., ¶ 42 – "Under Business & Professions Code §1680(z)(1)(C) a dentist is required to report to the Medical Board the removal of any patient to whom general anesthesia to a hospital or emergency center for medical treatment. Dr. Sadek could avoid this reporting requirement by not allowing the EMS to transport Amelia Reilly when she became bradycardic the first time."
5. Declaration of Joseph Renzi, D.D.S., ¶ 43 – "The lack of immediate and consistent CPR from Dr. Sadek or his dental assistant, who were both Pediatric Advanced Life Support (PALS) certified providers, was egregious and wanton, in conscious disregard for the health and safety of Amelia Reilly."
6. Declaration of Joseph Renzi, D.D.S., ¶ 45, Page 7:13-15 – "was reckless" and "conscious disregard for her health and safety."
7. Declaration of Joseph Renzi, D.D.S., ¶ 47, Page 7:19-21 – "Based upon my education, training, experience, and my review of all of the information, it is my opinion that Dr. Pham's actions and inactions constituted a wanton disregard of the rights and safety of this five year-old child."

Sadek and PCD's objections are otherwise OVERRULED.

Plaintiffs' Evidentiary Objections – Plaintiffs object to the following evidence submitted by Sadek and Pham in support of their Oppositions:

1. Declaration of Thomas Einstein, M.D., ¶ 10, Page 3:19-22 – "Based on the information available, there is an excellent chance the cardiologist would have cleared the patient for the procedure as had occurred previously."

2. Declaration of Thomas Einstein, M.D., ¶ 17, Page 5:1-3 - “Dr. Pham, Dr. Sadek and the paramedics all endorsed the decision not to transport. The paramedics did not even document the visit, and we do not know if they consulted with a physician at the emergency department.”

3. Declaration of Thomas Einstein, M.D., ¶ 17, Page 5:3-5 - “at the time all present believed the danger had passed and that this patient would do well. Not one person present suggested otherwise.”

4. Declaration of Thomas Einstein, M.D., ¶ 22, Page 6:7-11 - “Dr. Pham prioritized patient safety over maintaining his income. . . . The time taken to make this decision and the plan going forward demonstrate a thoughtful approach to the well-being of patients in spite of the economic cost.”

5. Declaration of Thomas Einstein, M.D., ¶ 23, Page 6:12-20 - “it is my opinion that Dr. Pham’s conduct on January 15, 2025, was most certainly not egregious, wanton, despicable, and reckless. The records, testimony and video fail to demonstrate any malice or ill intent towards this patient or any desire to cause her harm. . . . [T]hose judgment calls and decisions were not made with the expectation that this patient would suffer any ill effects as the extreme allegations would incorrectly assert.”

Plaintiffs argue certain portions of the Einstein Declaration lack qualification, constitute speculation, and there is an insufficient basis for expert opinion pursuant to Evidence Code sections 720, 801(b), 802, and 803 because Einstein is not a cardiologist and does not identify any cardiology training, education, or experience to qualify him to predict whether a pediatric cardiologist would have cleared Amelia for anesthesia. They also object on the grounds that certain statements assume facts not in evidence, lack foundation, speculates concerning others’ subjective beliefs and motive which Einstein has no personal knowledge of, constitute improper expert opinion, are irrelevant, and constitute improper legal conclusions.

6. Declaration of Yasser Sadek, D.D.S., ¶ 6, Page 3:11 – “At no time did I consciously disregard a known risk to Amelia.”

7. Declaration of Yasser Sadek, D.D.S., ¶ 8, Page 4:7-8 - “After both parents represented that Amelia was healthy and did not have an active medical condition requiring special precautions.”

8. Declaration of Yasser Sadek, D.D.S., ¶ 15, Page 6:13-14 - “Decisions concerning transport were made by EMS personnel based upon their own assessment.”

9. Declaration of Yasser Sadek, D.D.S., ¶ 16, Page 6:19-24 - “which is consistent with the representations made by both Mr. and Mrs. Reilly to this Declarant that, as far as they were concerned, their daughter was healthy.”

As to the Sadek Declaration, Plaintiffs object on the grounds that certain portions constitute improper legal conclusions, constitute inadmissible hearsay and improper lay medical opinion under Evidence Code sections 800 and 1200, lack personal knowledge and foundation, and are speculative.

Plaintiffs’ objections are SUSTAINED in part as to the following specific language:

1. Declaration of Thomas Einstein, M.D., ¶ 10 – “Based on the information available, there is an excellent chance the cardiologist would have cleared the patient for the procedure as had occurred previously.”

2. Declaration of Thomas Einstein, M.D., ¶ 17 – “Dr. Pham, Dr. Sadek and the paramedics all endorsed the decision not to transport. The paramedics did not even document the visit, and we do not know if they consulted with a physician at the emergency department.”

3. Declaration of Thomas Einstein, M.D., ¶ 17 – “at the time all present believed the danger had passed and that this patient would do well. Not one person present suggested otherwise.”

4. Declaration of Thomas Einstein, M.D., ¶ 22 – “Dr. Pham prioritized patient safety over maintaining his income.”

5. Declaration of Thomas Einstein, M.D., ¶ 23 – “it is my opinion that Dr. Pham’s conduct on January 15, 2025, was most certainly not egregious, wanton, despicable, and reckless. The records, testimony and video fail to demonstrate any malice or ill intent towards this patient or any desire to cause her harm. . . . [T]hose judgment calls and decisions were not made with the expectation that this patient would suffer any ill effects as the extreme allegations would incorrectly assert.”

6. Declaration of Yasser Sadek, D.D.S., ¶ 8 – “After both parents represented that Amelia was healthy and did not have an active medical condition requiring special precautions.” – Sustained only to the extent the statement is offered to establish Amelia’s medical fitness for office anesthesia. Overruled to the extent the statement reflects the basis for Sadek’s personal state of mind.

7. Declaration of Yasser Sadek, D.D.S., ¶ 15 – “based upon their own assessment.”

8. Declaration of Yasser Sadek, D.D.S., ¶ 16 – “which is consistent with the representations made by both Mr. and Mrs. Reilly to this Declarant that, as far as they were concerned, their daughter was healthy.” Sustained only to the extent the statement is offered to establish Amelia’s medical fitness for office anesthesia or cause of death. Overruled to the extent the statement reflects the basis for Sadek’s personal state of mind.

Plaintiffs’ objections are otherwise OVERRULED.

III. Legal Standard

Standard for Leave to Amend to Allege Punitive Damages Against a Healthcare Provider – Motions for leave to amend are generally governed by Code of Civil Procedure section 473, subdivision (a). However, the complaint in “any action for damages arising out of the professional negligence of a health care provider” cannot include a claim for punitive damages unless “the court enters an order allowing an amended pleading.” (Code Civ. Proc., § 425.13, subd. (a).) Section 425.13 was enacted “amid concern over routine inclusion of sham punitive damage claims in medical malpractice actions,” and aims to alleviate the problem by “shifting to the plaintiff the procedural burden that would otherwise fall on the defendant to remove a ‘frivolous’ or ‘unsubstantiated’ claim early in the suit.” (College Hospital Inc. v. Superior Court (1994) 8 Cal.4th 704, 717.)

The Court may allow the filing of an amended pleading claiming punitive damages on a motion by the party seeking leave to amend and on the basis of the supporting and opposing affidavits presented that the plaintiff has established that there is a substantial probability that the plaintiff will prevail on the claim pursuant to Civil Code section 3294. (Code Civ. Proc., § 425.13, subd. (a).) The motion must be filed within two years after the complaint or initial pleading is filed or no less than nine months before the date the matter is set for trial, whichever is earlier. (Code Civ. Proc., § 425.13, subd. (a).)

A section 425.13 motion “operates like a demurrer or motion for summary judgment in ‘reverse.’” (College Hospital, *supra*, 8 Cal.4th at 719.) “Rather than requiring the defendant to defeat the plaintiff’s pleading by showing it is legally or factually meritless, the motion requires the plaintiff to demonstrate that he possesses a legally sufficient claim which is ‘substantiated,’ that is, supported by competent, admissible evidence.” (Ibid.)

"[I]t is only necessary that plaintiff provide 'a sufficient prima facie showing of facts to sustain a favorable decision if the evidence submitted by the [plaintiff] is credited.' [Citation.] The trial court is not required to make any factual determination or to become involved in any weighing process beyond that necessarily involved in deciding whether a prima facie case for punitive damages exists. Once the court concludes that such a case can be presented at trial it must permit the proposed amended pleading to be filed. If it concludes that no such case exists, then it properly rejects the proposed pleading amendment. In making this judgment, the trial court's consideration of the defendant's opposing affidavits does not permit a weighing of them against the plaintiff's supporting evidence, but only a determination that they do not, as a matter of law, defeat that evidence." (Looney v. Superior Court (1993) 16 Cal.App.4th 521, 539.) "[A] prima facie case of entitlement to punitive damages must meet the standard that plaintiffs will be likely to prevail by showing malice, oppression, or fraud by clear and convincing evidence." (Aquino v. Superior Court (1993) 21 Cal. App. 4th 847, 854-55.)

In ruling on a Section 425.13 motion, the trial court "should not engage in weighing conflicting evidence, making credibility determinations, or drawing inferences from facts in assessing whether a plaintiff has made a sufficient showing of a prima facie case of entitlement to punitive damages. [Citation.]" (Id. at 856.) In considering the affidavits and evidence provided by the party opposing the motion, "any uncontradicted evidence presented in opposition should be considered to fill in any blank areas in the plaintiff's showing, in order to give the court as complete a factual picture as possible," and "[t]he court must also take care to distinguish between evidentiary facts and argument or hyperbole presented by counsel." (Ibid.) The Court must view the evidence in the light most favorable to the plaintiff. (Id. at 857.)

Standard for Recovery of Punitive Damages – Punitive damages may be recovered upon a proper showing of malice, fraud, or oppression. (Civ. Code, § 3294, subd. (a).) "Malice" is defined as conduct intended to cause injury to a person or despicable conduct carried on with a willful and conscious disregard for the rights or safety of others. (Turman v. Turning Point of Cent. Cal., Inc. (2010) 191 Cal.App.4th 53, 63.) "Oppression" means despicable conduct subjecting a person to cruel and unjust hardship, in conscious disregard of the person's rights. (Ibid.) "Fraud" is an intentional misrepresentation, deceit, or concealment of a material fact known by defendant, with intent to deprive a person of property, rights or otherwise cause injury. (Ibid.) Conclusory allegations, devoid of any factual assertions, are insufficient to support a conclusion that parties acted with oppression, fraud or malice. (Smith v. Superior Court (1992) 10 Cal.App.4th 1033, 1042.)

IV. Discussion

Application – Plaintiffs move for leave to amend their FAC to add a claim for punitive damages against Defendants pursuant to Code of Civil Procedure sections 425.13, 473, and 576, and Civil Code section 3294. (Notice of Motion, p. 2:2-5.) Specifically, Plaintiffs seek to allege punitive damages as to the third cause of action for negligence, the fourth cause of action for lack of informed consent, the fifth cause of action for battery, the sixth cause of action for negligent misrepresentation, the seventh cause of action for intentional misrepresentation, the eighth cause of action for fraud by concealment, and the ninth cause of action for constructive fraud.

To prevail on the motion, Plaintiffs must, through competent and admissible affidavits, make a prima facie showing by clear and convincing evidence that there is a substantial probability that they will prevail on their punitive damages claim as to each Defendant. Plaintiffs allege Defendants acted with malice and fraud, thereby subjecting them to punitive liability.

"Malice" includes "despicable conduct which is carried on by the defendant with a willful and conscious disregard of the rights or safety of others." (Civ. Code, § 3294, subd. (c)(1).) Conscious disregard for the safety of others may constitute malice required to sustain a claim for punitive damages, and exists where the defendant is aware of probable dangerous consequences and willfully and deliberately fails to avoid them. (Blegen v. Superior Court (1981) 125 Cal.App.3d 959, 962-63.) Malice may be inferred from the circumstances of a defendant's conduct. (Monge v. Superior Court (1986) 176 Cal.App.3d 503, 511.) As with recklessness, a jury may infer conscious disregard where the flaws in a defendant's policies or conduct are so obvious that the defendant can be found to have essentially turned a blind eye to a high probability of harm. (Samantha B. v. Aurora Vista Del Mar, LLC (2022) 77 Cal.App.5th 85, 100.)

As to fraudulent concealment, Plaintiffs must show “(1) concealment or suppression of a material fact; (2) by a defendant with a duty to disclose the fact; (3) the defendant intended to defraud the plaintiff by intentionally concealing or suppressing the fact; (4) the plaintiff was unaware of the fact and would have acted differently if the concealed or suppressed fact was known; and (5) the plaintiff sustained damage as a result of the concealment or suppression of the material fact.” (Rattagan v. Uber Technologies, Inc. (2024) 17 Cal. 5th 1, 40.) Doctors and healthcare providers have a duty to disclose to their patients “all material information that is necessary to make an informed decision about a proposed treatment” and a separate duty “not to misrepresent the nature of a patient’s medical condition.” (Hahn v. Mirda (2007) 147 Cal.App.4th 740, 749.)

i. Defendant Sadek

As against Sadek, the evidence presented and factual allegations pled by Plaintiffs show the following:

Plaintiffs were referred by their dentist, Dr. Kanarki, to PCD, called Palmdale Children’s Dental, because it was focused on treating children. (Mrs. Reilly Decl., ¶ 3.) PCD held itself out as providing pediatric care on its website and by nature of its name, identified Sadek as a pediatric dentist, stated on its website that IV sedation is a “good option” for children with special needs and complicated cases, and Sadek accepted Amelia as a pediatric patient with special medical needs. (Ex. 9, Sadek Depo., p. 26:18–27:5, 46:20–47:18, 132:1–133:23; Ex. 10.) Sadek testified that he did not obtain any additional formal education or training in pediatric dentistry after four years of dental school, has no board certification with the American Academy of Pediatric Dentistry or the American Society of Dentistry for Children, has not taken any courses on administration of general anesthesia to pediatric patients, and admits he is not a pediatric dentist as was advertised on the PCD website, but a general dentist, limiting practice to pediatrics. (Ex. 9, Sadek Depo., p. 56:3–14, 94:6–95:18.) Plaintiffs assert that they believed Sadek was a pediatric dentist, and would not have allowed Amelia to receive dental treatment with Sadek or PCD had they known he was not a pediatric dentist. (Mrs. Reilly Decl., ¶¶ 3-4; Mr. Reilly Decl., ¶¶ 3-4.)

Mrs. Reilly declares that Sadek did not discuss treatment options other than nitrous oxide or IV sedation, did not inform her Amelia could be referred to another provider for an alternative approach to treatment, and did not know that Williams Syndrome could increase anesthesia risk due to cardiac complications. (Mrs. Reilly Decl., ¶¶ 5-6.) Mrs. Reilly also states that Sadek never informed her that he did not understand the medical implications of Williams Syndrome, that he did not know whether it involved cardiovascular abnormalities, that he had not researched it before recommending anesthesia, that he had not reviewed Amelia’s medical or cardiology records, that he had not determined whether Amelia was an appropriate candidate for anesthesia given her cardiac history and diagnoses, or that cardiology clearance was required. (Mrs. Reilly Decl., ¶ 7.)

Before anesthesia, Sadek knew Amelia had Williams Syndrome and a heart murmur, but did not understand what Williams Syndrome is and did not look it up prior to recommending anesthesia. (Ex. 5; Ex. 9, Sadek Depo., p. 16:13-20, 132:21–133:23; Ex. 10, p. 2, 4; Ex. 13, Mrs. Reilly Depo., p. 36:9–15.) After prior treatment with nitrous oxide failed, Sadek recommended IV sedation to Mrs. Reilly beginning in November 2024 and again on December 13, 2024. (Ex. 9, Sadek Depo., p. 149:24-150:12; Ex. 13, Mrs. Reilly Depo., p. 44:4-8, 46:17-47:9.) Mrs. Reilly testified that she asked Sadek whether IV sedation was the only route and whether she needed to contact Amelia’s cardiologist, and Sadek answered that IV sedation was the only option and said, “No. This is a fast-and-quick procedure. It will be done in 30 minutes.” (Ex. 13, Mrs. Reilly Depo., p. 46:17–47:9.)

Amelia previously received general anesthesia in 2023 and at that time was classified as ASA Class IV.. (Ex. 5, p. 109.) Sadek testified in his deposition that Mrs. Reilly never mentioned that Amelia was being treated by a cardiologist, never asked whether a cardiologist needed to be contacted, and that he discovered that she had a cardiologist approximately 10 minutes before the end of the procedure, discovered that she previously received surgery after the procedure, and that he would have referred Amelia to a children’s hospital rather than establishing her as a patient with PCD if he had known she was under cardiologist care. (Ex. 9, p. 143:9-145:1.) Sadek testified that he has a general idea about ASA classifications, understands that ASA 4 is an unhealthy child, and testified that “there is absolutely no way we were treating anyone with ASA 3 or 4” and that to his knowledge, all patients treated by PCD were healthy enough to be treated. (Ex. 9, p. 66:4-67:13.) Sadek also testified to having reviewed portions of Amelia’s patient registration form with PCD prior to recommending anesthesia, including family and medical history check boxes on the form, but no other medical records. (Ex. 9, p. 132:1-133:23.) After the procedure, Sadek left the procedure room and went to his office, shortly afterwards a DA informed Sadek that Amelia’s heart rate dropped, and Sadek called paramedics on his way back to the procedure room. (Ex. 9, p.

173:19-174:4.)

Mr. Reilly testified that after the procedure, Sadek informed him that he replaced a tooth and filled additional teeth for which Mr. Reilly did not consent to treatment, stated that Amelia's heart rate had dropped after the procedure was complete, but "everything is okay now," and informed him that he had called EMS for precautionary measures. (Ex. 14, p. 43:1-11.) Plaintiffs also submit a transcript of the video collected during the incident, which establishes that on the first cardiac crash, Sadek called 911 and mentioned something about Amelia's heart rate, provided the address and suite to PCD, stated Amelia has Williams Syndrome after it was mentioned by Pham, that Amelia was awake and her heart rate was back, and that they were waking her up. (Ex. 7, p. 12:8-16:23.)

After EMS arrived, Pham sent the paramedics away, discussed infra, and Sadek testified that he left the decision up to Pham as to whether to send Amelia to the hospital, but that he never had a conversation with anyone whether it would be a good idea to send her to the hospital. (Ex. 9, p. 231:7-23.) Shortly thereafter, Amelia's heart rate crashed again, and Sadek called 911 a second time and asked paramedics to return. (Ex. 7, p. 40:13-41:22.) Amelia was transported by ambulance to Antelope Valley Hospital and underwent approximately one hour of resuscitative efforts before she was pronounced dead. (Ex. 13, Ms. Reilly Depo. 69:12-15.)

Plaintiffs also submit the Declaration of Joseph Renzi, Jr., D.D.S., a board-certified pediatric dentist who has treated thousands of children with and without nitrous oxide, oral sedation, and IV sedation. (Renzi Decl., ¶¶ 1-2.) Renzi states a board-certified pediatric dentist must complete substantially more training than a general dentist, including an accredited pediatric dentistry residency, which is generally two years, with child-specific training, including training specific to children with congenital syndromes, special needs dentistry, recognition of medical conditions requiring consultation, treatment of child patients with special health care needs, sedation and anesthesia, emergency management, and management of medically complex children, and must then pass oral and written examinations administered by the American Board of Pediatric Dentistry. (Renzi Decl., ¶¶ 13-14.) Renzi notes that Business and Professions Code section 1646.3, subdivision (a) requires a physical evaluation and medical history be taken prior to general anesthesia, that the Dental Board regulations require maintenance of medical history, physical evaluation, deep sedation, and general anesthesia records, and that the regulations require patient-specific records documenting ASA classification, known or suspected medically-compromising conditions, rationale for anesthesia, airway evaluation, and heart and lung assessment pursuant to California Code of Regulations, title 16, section 1043.3, subdivision (b)(1).

Renzi opines that for a five year old child with Williams Syndrome, documented cardiac history, and prior anesthesia records classifying her as ASA IV, patient-specific screening, cardiology clearance, review of prior anesthesia and cardiology records, and proper location in a hospital were central safety issues prior to administration of anesthesia, and that Sadek failed to comply with threshold safety issues in his treatment of Amelia. (Renzi Decl., ¶¶ 20-21.) Renzi declares that Sadek failed to meet these requirements for a number of reasons, including that Sadek (1) accepted Amelia as a patient despite lacking the knowledge to safely evaluate a child with Williams Syndrome, (2) assured Mrs. Reilly that IV sedation was the only option and no clearance was necessary due to the brevity of the procedure, which is untrue, (3) stated nitrous oxide and sedation are used because they are safer than local anesthesia, which is not true, (4) did not consult with Amelia's pediatric cardiologist or pediatrician before recommending and using general anesthesia on her, (5) administered anesthesia despite being unable to reasonably determine Amelia's suitability because he never obtained or reviewed her records or obtained cardiologist clearance, (6) administered anesthesia despite Amelia not being an appropriate candidate without pediatric cardiology consultation and review of her prior records, (7) failed to refer Amelia for treatment in a hospital setting, (8) failed to inform Plaintiffs of the increased anesthesia risks associated with Williams Syndrome prior to the procedure, and (9) failed to remain present throughout the duration of the procedure and assist in emergency response. (Renzi Decl., ¶¶ 26-28, 30-31, 33-35, 38-39.)

Renzi also declares that the videographic evidence shows Sadek left the room multiple times during the procedure, that only Pham was present in the room during the first cardiac collapse, that the standard of care required Sadek to send Amelia with EMS to be transported to the hospital but allowed her to remain in his office, and that Sadek did not start CPR during the second cardiac collapse, but stood with his hands in his pockets until he was instructed to act, along with his DAs. (Renzi Decl., ¶¶ 39-43.) Renzi ultimately opines that to a reasonable degree of medical probability, had Sadek properly screened Amelia, obtained and reviewed her prior records, and consulted with her pediatric cardiologist or pediatrician before recommending anesthesia, Amelia would not have been approved for general anesthesia in Sadek's office, the anesthesia would not have proceeded, and Amelia would not have passed away. (Renzi Decl., ¶ 48.)

In opposition, Sadek declares that he routinely relies on information supplied by a child's parents or guardians together with information contained in the patient's health history and information obtained by the anesthesia provider, that he was aware Amelia had Williams Syndrome but did not personally perform the medical and anesthetic risk assessment for general anesthesia, relied on Pham's professional anesthetic assessment in proceeding with the dental treatment and believed she could safely undergo anesthesia, and that he asked Plaintiffs whether Amelia had any current medical conditions requiring special precautions and both represented that she was healthy. (Sadek Decl., ¶¶ 5-9.) The evidence also provides that following this incident, Sadek, Pham, and PCD ceased all anesthesia administration for a number of months and placed into effect specific protocols and procedures for all future treatment, including required direct contact with all patients' pediatricians and evaluation of prior medical records before clearing a child for general anesthesia.

The court must determine "whether the evidence presented is such that a jury applying that [clear and convincing] evidentiary standard could reasonably find" for the plaintiff. (*Hoch v. Allied-Signal, Inc.* (1994) 24 Cal.App.4th 48, 60.) However, "section 425.13(a) does not authorize the trial court to reject a well-pled and factually supported punitive damages claim simply because the court believes the evidence is not strong enough for probable success before a jury." (*College Hospital*, supra, 8 Cal.4th at 725-26.)

Plaintiffs must substantiate their claims "with 'factual recitals' [] 'made under penalty of perjury' and 'set forth competent admissible evidence within the personal knowledge of the declarant.'" [Citation.] *College Hospital* made clear that plaintiff's own pleading is entitled to no evidentiary weight in the analysis." (*Pomona Valley Hospital Medical Center v. Superior Court* (2013) 213 Cal.App.4th 828, 837.) On review of the moving papers and proposed SAC, the Court finds that Plaintiffs have sufficiently shown a prima facie case for punitive damages.

As to their fraudulent concealment theory for punitive damages, Plaintiffs must show "(1) concealment or suppression of a material fact; (2) by a defendant with a duty to disclose the fact; (3) the defendant intended to defraud the plaintiff by intentionally concealing or suppressing the fact; (4) the plaintiff was unaware of the fact and would have acted differently if the concealed or suppressed fact was known; and (5) the plaintiff sustained damage as a result of the concealment or suppression of the material fact." (*Rattagan v. Uber Technologies, Inc.* (2024) 17 Cal. 5th 1, 40.) Direct evidence of intent to defraud is not required. "[F]raudulent intent is an issue for the trier of fact to decide" (*Beckwith v. Dahl* (2012) 205 Cal.App.4th 1039, 1061), and "must often be established by circumstantial evidence." (*Tenzer v. Superscope, Inc.* (1985) 39 Cal.3d 18, 30.) The requisite intent to conceal or suppress can be inferred from the defendant's conduct and the surrounding circumstances. The heightened specificity ordinarily required to plead fraud applies to claims based on affirmative misrepresentations, not to claims of fraudulent concealment. (*Alfaro v. Community Housing Improvement System & Planning Assn., Inc.* (2009) 171 Cal.App.4th 1356, 1384.) Plaintiffs' concealment allegations therefore are not defeated by the absence of the particularity that would be demanded of a claim resting on an affirmative misrepresentation.

Plaintiffs have put forth sufficient evidence to establish a prima facie case of fraud sufficient to subject Sadek to punitive damages under their fraudulent concealment theory. Although there is no direct evidence of Sadek's intent to defraud, the record supplies circumstantial evidence from which a reasonable jury could infer that Sadek intentionally concealed or suppressed facts he was obliged to disclose. Plaintiffs have shown that Sadek had no basis for his assurance that no cardiologist need be contacted prior to general anesthesia, that he had a duty to disclose the absence of any such basis, that Plaintiffs were unaware of that fact and would have acted differently had they known of Sadek's lack of information and knowledge, and that they sustained damages. From the same evidence, a jury could reasonably infer the intent element: that Sadek, aware he had no basis for his categorical safety assurance, withheld both that lack of basis and his corresponding failure to investigate in order to obtain Plaintiffs' consent and to proceed with the anesthesia and procedure. The same inference is available as to Sadek's failure to disclose his unfamiliarity and inexperience with Williams Syndrome, his non-review of medical records from Amelia's other medical providers, the characterization of PCD as a practice equipped to treat children with complex or special medical needs, and the availability of referral to another provider for alternate treatment plans unavailable at PCD.

Plaintiffs have also met their burden as to malice. "'Malice' means conduct which is intended by the defendant to cause injury to the plaintiff or despicable conduct which is carried on by the defendant with a willful and conscious disregard of the rights or safety of others." (Civ. Code, § 3294, subd. (c)(1).) The statute plainly indicates that absent an intent to injure the plaintiff, 'malice' requires more than a 'willful and conscious' disregard of the plaintiffs' interests, as the additional component of 'despicable conduct' must be found. (See, e.g., *Angie M. v. Superior Court* (1995) 37 Cal.App.4th 1217, 1221-1222.) "The

adjective 'despicable' connotes conduct that is...so vile, base, contemptible, miserable, wretched or loathsome that it would be looked down upon and despised by ordinary decent people." (Lackner v. North (2006) 135 Cal.App.4th 1188, 1210.) "The mere carelessness or ignorance of the defendant does not justify the imposition of punitive damages.... Punitive damages are proper only when the tortious conduct rises to levels of extreme indifference to the plaintiff's rights, a level which decent citizens should not have to tolerate." (Ibid.)

The sworn evidence presented by Plaintiffs and factual allegations in the proposed SAC do not establish or allege that Sadek intended to injure Amelia, and therefore, Plaintiffs must establish both conscious disregard and despicable conduct beyond mere commission of a tort. Plaintiffs' evidence establishes that Sadek accepted Amelia as a patient with knowledge of her Williams Syndrome diagnosis but did not have personal experience with or knowledge of Williams Syndrome, that he was not specially trained to identify risks of Williams Syndrome as a general dentist, that he made no effort to contact Amelia's pediatrician or other healthcare provider to review her medical history, that despite his awareness of her Williams Syndrome and heart murmur, made no effort to look into Williams Syndrome to confirm whether any additional clearance, research, testing, or precautionary measures were necessary before recommending and moving forward with the procedure, and that he never disclosed to Plaintiffs that he had no knowledge of or experience with Williams Syndrome despite advertising his dental practice as one targeted for children with special needs or complex medical cases, a representation that Plaintiffs allegedly relied on in seeking treatment from Sadek and PCD.

It also shows that PCD primarily treats children, that Sadek's website advertises treatment of special needs children and complicated cases, states Sadek is a pediatric dentist, that where a documented condition in a patient's chart was not known to Sadek to present complications or potential issues with anesthesia, he would not look further into it before authorizing administration of anesthesia, and that in Amelia's case, he relied solely on Plaintiffs to disclose any and all potential medical issues or complications rather than verifying the information with Amelia's medical providers or conducting any inquiry into her conditions as a licensed medical provider. It shows that Sadek represented to Mrs. Reilly that no cardiology intervention or clearance was required and that the procedure was safe, prior to Pham assigning an ASA II classification and despite having no basis for those claims, that Sadek left the room during the procedure and shortly thereafter while Amelia should have been monitored and Sadek was under a legal obligation to remain present and assist in any emergency response, that he performed additional dental work on Amelia beyond the scope consented to by Plaintiffs, including work on at least four additional teeth, took no action while Pham dismissed EMS despite Amelia's unstable condition, allowed the monitoring devices to be removed shortly after Amelia's first emergency, and failed to adequately intervene and begin emergency life-saving treatment.

While Sadek puts forth his declaration refuting many of these assertions and clarifies the standards and procedures used in his office, the weighing of the evidence and credibility determinations are not the job of the Court, but of the jury. Therefore, while Sadek's assertions may well be true and he may have viable defenses to these punitive damages claims, the evidence of Plaintiffs, which the Court must credit, meets Plaintiffs' burden.

Therefore, Plaintiffs have sufficiently stated and demonstrated a triable punitive damages claim, which warrants placing the issue in front of a jury.

Accordingly, Plaintiffs' motion is GRANTED as to Sadek.

ii. Defendant PCD

As against PCD, Plaintiffs primarily assert punitive damages are warranted because Sadek was the sole shareholder and Chief Executive Officer, and that his decisions concerning patient selection, office anesthesia, cardiology referral, treatment options, and emergency escalation were made within the practice he owned and controlled, and the punitive showing against Sadek therefore supports punitive liability against PCD. (Motion, p. 26:13-19; Ex. 9, Sadek Depo. 44:2-25.)

A corporate employer is liable for punitive damages when an officer, director, or managing agent personally commits, authorizes, or ratifies the fraud, oppression, or malice. (Civ. Code, § 3294, subd. (b).) "An act of oppression, fraud or malice, by an officer, director or managing agent, is sufficient to impose liability on a corporate employer for punitive damages,

without any additional showing of ratification by the employer.” (Kelly-Zurian v. Wohl Shoe Co. (1994) 22 Cal.App.4th 397, 420.) A managing agent includes “only those corporate employees who exercise substantial independent authority and judgment in their corporate decisionmaking so that their decisions ultimately determine corporate policy.” (White v. Ultramar, Inc. (1999) 21 Cal.4th 563, 566-67.) For an employer to be liable for punitive damages, “such conduct must have been performed by an ‘agent ... employed in a managerial capacity and ... acting in the scope of employment,’ ” or ratified or approved by a ‘managerial agent’ of the organization” and “in performing, ratifying, or approving the malicious conduct, the agent must be acting as the organization’s representative, not in some other capacity.” (College Hospital, supra, 8 Cal.4th at 723.)

Here, as discussed, Plaintiffs have stated and sufficiently demonstrated that they have a triable claim for punitive damages as to Sadek, the CEO of PCD. Further, as pled and substantiated, Plaintiffs have sufficiently established that (1) Sadek, as CEO, is a managing agent of PCD, and (2) Sadek was acting in the course and scope of his employment at all relevant times of his alleged conduct.

Accordingly, Plaintiffs’ motion is GRANTED as to PCD.

iii. Defendant Pham

As against Pham, along with the relevant evidence discussed above as to Sadek, the allegations in the proposed SAC and evidence provided by Plaintiff establishes the following:

Dr. Pham was the practice’s anesthesia provider. (Ex. 9, p. 74:7-13, 137:22-138:3; Ex. 11, Pham Depo., p. 166:3-25.) Pham knew Amelia had Williams Syndrome and a heart murmur prior to administration of anesthesia. (Ex. 11, Pham Depo. 166:3–25.) Before anesthesia, Mr. Reilly told Pham that Amelia had Williams Syndrome and a heart murmur. (Ex. 14, Mr. Reilly Depo., p. 25:4–26:1.) Pham searched Williams Syndrome on Google on his phone and learned that Williams Syndrome presents an increased risk of anesthesia where cardiac issues are present. (Ex. 11, Pham Depo, p. 50:10-52:10.) Pham asked Mr. Reilly whether Amelia had cardiac issues and was informed that she received surgery eight months prior to the dental procedures and the surgeon and anesthesiologist had no concerns. (Ex. 11, p. 52:11-53:8.) At the time he administered the anesthesia, Pham was not aware of any precautions that needed to be taken concerning administration to children with Williams Syndrome, and did not have a general understanding of what Williams Syndrome was, associated risks, or how it manifested itself. (Ex. 11, p. 53:9-18.) During and between Amelia’s cardiac crashes, Pham stated that her heart was acting weird, reminded Sadek that she had Williams Syndrome while on the phone with 911, and informed Plaintiffs that he would not perform general anesthesia again on Amelia because while cardiac crashes may be fixed the first time, that does not mean it can be fixed a second time. (See Ex. 7.)

Pham did not customarily review his patients’ dental charts, and relied on the parents to provide their medical history. (Ex. 11, p. 53:19-54:10.) Pham classified Amelia as ASA II based on Mr. Reilly’s representations, but never reviewed any of her medical records, including those from Children’s Hospital associated with her prior surgery that classified her as ASA IV, and video evidence of the procedure shows that Pham did not conduct the necessary examination of Amelia’s heart before determining she was ASA Class II. (Ex. 11, p. 86:14-87:3.) Pham indicated in his record as an estimate that he gave 25 milligrams of Ketamine and 30 milligrams of Propofol to Amelia, but on his chart he indicated that he gave 15 milligrams of each. (Ex. 11, p. 92:5-10.)

When EMS arrived after Amelia’s first cardiac crash, Pham said Amelia was “good now” and informed EMS that they called to have backup if needed, but their assistance was no longer needed. (Ex. 7, p. 15:20–16:23; Ex. 8, p. 23.) Pham kept Amelia in the office, but told her parents that if Amelia’s heart rate fell or stopped, “we only have minutes,” and indicated that the same event could be treated faster and without paramedic intervention in a hospital. (Ex. 7, p. 20:8–24:9.) After EMS left, Pham removed Amelia’s IV, extubated her, and instructed the DA to remove the ECG leads and blood-pressure cuff, resulting in delayed recognition that she was again experiencing a medical emergency. (Ex. 11, p. 124:3-125:7, 132:15-18.) Mr. Reilly asked Pham about Amelia’s shallow breathing, and Pham responded that she was “breathing real good.” (Ex. 7, p. 34:6-36:13.) On Amelia’s second cardiac crash, Pham recognized that the IV had been removed and testified that the missing IV delayed epinephrine by approximately one to two minutes. (Ex. 11, p. 134:4-10.) The anesthesia record does not disclose that the IV was removed and reestablished during the second emergency. (Ex. 11, p. 134:4–10, 164:11–17.)

Mr. Reilly submits a declaration, stating that Pham never informed him that Williams Syndrome with cardiac issues increased the risk of complications from anesthesia, that he had not contacted Amelia's cardiologist, reviewed her cardiology records, or obtained a pediatric cardiology clearance, or that he did not know that children with Williams Syndrome should receive general anesthesia only in a hospital setting. (Mr. Reilly Decl., ¶ 13.) He also declares that he understood from Pham's questions and decision to proceed as a medical determination, based on Amelia's history, that anesthesia could safely be administered in Sadek's office. (Mr. Reilly Decl., ¶ 11.)

Plaintiffs also submit the Declaration of James W. Tom, a board-certified anesthesiologist, who declares that several published articles indicate that ketamine and propofol are not recommended as general anesthesia induction agents for patients with Williams Syndrome, that the pediatric cardiac anesthesiologist did not use propofol or ketamine in Amelia's March 2023 surgery, and that Pham used both, which falls below the standard of care. (Tom Decl., ¶¶ 26-28.) Tom also opines that Williams Syndrome and heart murmurs are facts requiring further investigation before obtaining parental consent and administering general anesthesia, that Plaintiffs would not have known of Amelia's prior ASA classification such that they could disclose that information, and that Pham's ASA II classification was insufficiently based on only a cursory review. (Tom Decl., ¶¶ 29-31.) Despite not checking Amelia's heart with a stethoscope, Pham indicated that her cardiac system was within normal limits, constituting a false entry in the anesthesia records but breaching the standard of care in failing to perform a physical examination. (Tom Decl., ¶ 31.) Tom also asserts that Pham left the room to take a phone call, failed to properly monitor Amelia, removal of the monitoring devices resulted in delayed recognition of Amelia's second cardiac emergency as evidenced by video of Amelia becoming still and her feet turning pale, that Pham did not properly initiate an emergency response, administered epinephrine and glycopyrrolate, which only temporarily stabilize a patient, and breached the standard of care in turning paramedics away and representing that Amelia was stable. (Tom Decl., ¶¶ 34-37.) Tom also declares that Pham admits in his deposition testimony that he completed the anesthesia record after the emergency, and the record itself appears to contain continuing heart-rate and blood-pressure monitoring during a time period when the video shows that Pham had removed the ECG leads and BP cuff, indicating the entries were fabricated by Pham. (Tom Decl., ¶¶ 46-47.) After Amelia was pronounced dead and Pham returned from the hospital, Tom notes that Pham added a notation regarding "aortic stenosis" to the pre-anesthesia record, and that the anesthesia records created by Pham contained false and misleading information. (Tom Decl., ¶¶ 50, 52.) Tom ultimately opines that to a reasonable degree of medical probability, proper screening would have prevented administration of general anesthesia and Amelia would not have passed away. (Tom Decl., ¶ 54.)

In opposition, Pham provides the Declaration of Thomas Einstein, M.D. The Einstein Declaration largely attempts to refute the evidence set forth by Plaintiffs, rather than provide evidence that provides a complete factual picture or fill in gaps left by Plaintiffs' evidence. As it is improper to weigh such evidence, the Einstein Declaration is largely immaterial to the ruling on this motion. Pham also provides supplemental excerpts from Mr. Reilly's testimony indicating that the information contained on Pham's anesthesia report accurately reflects the information provided by Mr. Reilly regarding Amelia's medical history. (Ex. 7.) Pham also provides Amelia's Death Investigation Summary from the County of Los Angeles Department of Medical Examiner, which asserts that she had mild aortic stenosis, no grossly identifiable cardiac abnormalities, no significant histopathologic abnormalities, and that while the levels of ketamine, lidocaine, and nitrous oxide were within therapeutic range, there was a temporal relationship between the administration of anesthesia and her death, and the cause of death was determined to be complications of anesthesia administration with Williams Syndrome as a contributing condition. (Ex. 8.)

Through affidavit and attached evidence, Plaintiffs have established a prima facie showing that they have a triable punitive damages claim as to Pham on malice grounds. The conduct alleged and substantiated shows that Pham, an anesthesiologist, was made aware of a prior surgery and administration of anesthesia, made aware of Amelia's Williams Syndrome and heart murmur, Googled Williams Syndrome in Mr. Reilly's presence and learned that cardiac complications with anesthesia may arise in patients with Williams Syndrome, was informed by Mr. Reilly that Amelia had prior aortic stenosis, failed to review any of Amelia's medical records and relied solely on representations made by Mr. Reilly in deciding to proceed with anesthesia, and with knowledge of his own ignorance of the diagnosis and the likelihood of complications, nonetheless consciously decided to move forward with administration. It also shows that Pham had some knowledge that Williams Syndrome could have been a contributing cause to the first cardiac crash, as Pham reiterated her diagnosis on Sadek's first call to paramedics, that Pham administered only temporary stabilization medication and subsequently informed EMS that Amelia was doing fine, removed all but one monitoring device without confirming stability, and was on his phone during the procedure and left the room to take a phone call during the procedure, at which time he was responsible for closely monitoring Amelia's vital signs. Pham allegedly reassured Plaintiffs that the procedure was safe despite having no personal experience with Williams Syndrome and knowledge that cardiac events were more likely in patients with the diagnosis, and the records created by Pham were modified after Amelia's death and contain measured information of

Amelia's vital signs, despite there being no monitoring devices placed on Amelia at the time of recording, indicating inconsistencies in the anesthesia report and Amelia's actual medical status after the first cardiac crash.

These factual allegations, substantiated by affidavit, are sufficient to meet Plaintiffs' burden of establishing a triable claim for punitive damages based on malice, as these facts could render a finding of conscious disregard for Amelia's safety and despicable conduct on behalf of a healthcare provider, warranting placement of the issue in front of a jury.

Accordingly, Plaintiffs' motion is GRANTED as to Pham.

Plaintiffs may file their proposed SAC to assert punitive damages against all Defendants. The Court's findings herein are narrowly limited to the purpose of this motion, and no analysis or ruling herein shall be construed as a determination on the merits of Plaintiffs' punitive damages claims or Defendants' possible defenses thereto.

V. Conclusion

Plaintiffs Michael Reilly and Cassandra Reilly's Motion for Leave to File a Second Amended Complaint to Allege Punitive Damages is GRANTED. Plaintiffs shall file their Proposed Second Amended Complaint within five days of this Order.